A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

30 April 2026

Dr Jessica Jose
Maternal & Infant Wellbeing Melbourne

Dear Dr Jose,

Re: Ms Caitlyn Hammond
DOB: 17/02/1987 Phone: 0457 097 893
Address: 182 Ascot Vale Rd, ASCOT VALE, VIC, 3032
Email: caitlynhammond@gmail.com
Medicare: 3423547576 / 1

Summary

- **Chronic abdominal pain and altered bowel habit, likely secondary to visceral hypersensitivity.**
- **Arranged gastroscopy, colonoscopy, and intravenous iron infusion.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Caitlyn, thank you very much for referring her.

Symptoms and history of presenting complaint

Caitlyn has a long-standing history of what can be best described as irritable bowel syndrome, originally diagnosed approximately 15 years ago. However, her symptoms have significantly exacerbated following the birth of her second child via caesarean section one year ago. She describes a fluctuating bowel pattern characterised by alternating bouts of constipation and diarrhoea. These diarrhoeal events occur roughly once every fortnight and are often preceded by a heavy sensation in the abdomen, nausea, and "gripey" mid-abdominal pain.

She does not report any traditional "red flag" symptoms such as haematochezia, unintentional weight loss, or nocturnal motions. While she has noted some relief of symptoms of anxiety with the commencement of sertraline for anxiety, this hasn't yet translated to improvement of symptoms of the underlying abdominal discomfort and bloating.

Diet

Caitlyn follows a pescatarian diet and has limited her lactose intake for several years. She maintains a high-fibre intake, including daily oats and seeds, and has previously attempted a low-FODMAP diet; however, this provided only marginal improvement in her symptoms, specifically regarding occasional heartburn.

Past medical history and medications

Caitlyn's medical history is significant for anxiety, for which she commenced sertraline 50 mg daily approximately five weeks ago. She has undergone two caesarean sections and a minor foot procedure. Her current medications include sertraline and oral iron supplements, the latter initiated following a diagnosis of iron deficiency earlier this year. She has no known drug allergies.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

Social history, family history

Caitlyn works as a public servant in strategy and policy and lives with her partner and two young children. She does not smoke and consumes alcohol only occasionally. Her family history is notable for a maternal grandfather diagnosed with bowel cancer at age 85. There is no family history of coeliac disease or inflammatory bowel disease.

Examination

On physical examination, Caitlyn appeared well. Her abdomen was soft and non-tender, with no palpable masses or organomegaly. There were no stigmata of chronic liver disease, and she did not display any rashes suggestive of inflammatory conditions.

Impression and plan

The clinical picture is most consistent with visceral hypersensitivity and an exaggerated gastrocolic reflex, likely exacerbated by her underlying anxiety. While her symptoms are functional in nature, the presence of persistent iron deficiency anaemia and borderline coeliac serology necessitates formal endoscopic evaluation to exclude organic pathology.

I have recommended a gastroscopy and colonoscopy to exclude inflammatory bowel disease, coeliac disease, and microscopic colitis. Given her significant fatigue and suboptimal response to oral iron, I will also perform an intravenous iron infusion (Ferinject) at the time of her procedures.

If her investigations return a normal result, we will focus management on the gut-brain axis. I have discussed the roles of gut-directed hypnotherapy and the potential off-label use of neuromodulators to down-regulate her visceral sensitivity. We also touched upon a trial of rifaximin to address her bloating symptoms.

I will review Caitlyn with her results in due course.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065